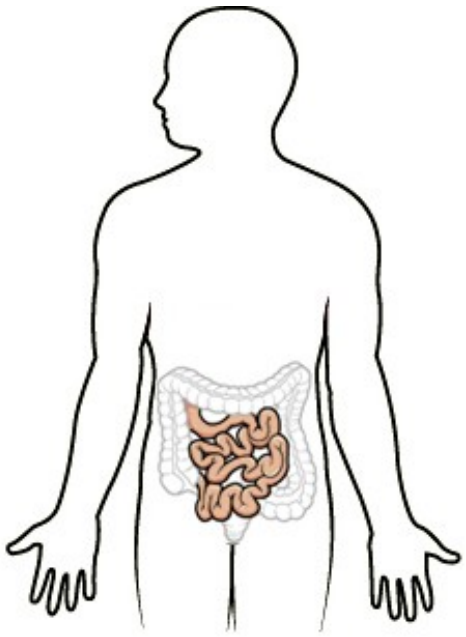
Small bowel resection is usually perfored for patients with intestinal obstruction, Crohn's disease, volvulus, intussuception, tumors, and abdominal trauma. The surgery may be performed via a transverse or vertical incision with the patient in the supine position. Once the section of small intestine to be excised has been identified and each end secured with bowel clamps, the small bowel is resected. The ends can be reanastamosed by a variety of surgical and stapling techniques. Blood loss is usually around 50-100 mL and the procedure can take between one and three hours. General anesthesia is employed, although an epidural is often placed for postoperative pain control. Because hypotension is often present due to sequestration of fluid in the abdomen, diarrhea, vomiting, and/or a prolonged NPO status, ketamine and etomidate are the perferred intravenous induction agents for this procedure.



Depending upon the reason for the surgery, a host of anesthetic implications must be considered. Patients with severe abdominal pathology may exhibit respiratory insufficiency, a decreased functional residual capacity, hypoxemia, and an increased A-a gradient due to pain or impairment of diaphragmatic excursion due to ascites, blood in the abdominal cavity, a tumor, or bowel distension. These patients are at risk for aspiration and should be treated as having a 'full stomach'. A rapid-sequence induction with cricoid pressure is standard and pretreatment with H2-receptor antagonists and sodium citrate should be considered. If the patient has a possible small bowel obstruction, metoclopramide is contraindicated because the increased peristalsis against the obstruction could result in perforation of the bowel.

Jaffe RA, Samuels SI. Anesthesiologist's Manual of Surgical Procedures. 4th ed. Philadelphia, PA: Lippincott Williams and Wilkins, 2009: 119.

Copyright 2014 Prodigy Anesthesia, Inc. It is unlawful to copy, distribute, and/or sell this material.